

A High-Support Autistic Person in Your Care

For family doctors, care-home GPs & clinicians — caring for an autistic adult in **assisted / residential / high-support care** (any age).

The one idea

Autism, through **monotropism**, means attention locks into a few deep, intense “tunnels.” For a high-support autistic person, care environments are often the **hardest of all**: sensory-hostile, routine-disrupting, full of unpredictable touch and demand. Many communicate without speech (AAC, behaviour, or a trusted other) and may not register or report pain until severe. Consent, capacity and dignity hinge on working *with* the person’s attention, routines and trusted people — never assuming incapacity from atypical communication.

What you may see — and what’s really going on

You may see...	What’s often happening
Distress, self-injury, “challenging behaviour,” refusal	Overwhelmingly communication about unmet need — pain, fear, sensory overload, lost routine
Meltdown or shutdown during care	Involuntary nervous-system response to overload — <i>not</i> aggression or non-compliance
Doesn’t report pain; sudden collapse / late presentation	Interoception differences — internal signals may not register until severe
Withdrawal when objects, order or staff change	Routines and possessions are identity and safety anchors
No reliable speech; relies on AAC, behaviour or a supporter	Atypical communication ≠ lack of capacity or understanding

Try this

- **Build care around predictability:** consistent staff, same sequence, visual schedules, advance warning of every change.
- **Lower sensory load** — quiet room, dim light, reduce beeps and strong smells; preserve the person’s own objects, interests and routines.
- **Investigate pain and physical causes first** for any new distress or behaviour change — constipation, dental pain, infection, ill-fitting equipment are common, hidden culprits.
- **Use supported decision-making:** explain in an accessible, literal form; allow processing time; consult the trusted supporter and any advance/communication profile.

- **Have a calm, written meltdown/shutdown plan:** reduce stimulation, ensure safety, give time and space — do not restrain or punish.

Avoid this

- Labelling distress “behavioural” before excluding pain and overload.
- Using restraint or sedation as a first response to meltdown/shutdown.
- Removing routines, objects or interests “for their own good,” or assuming lack of capacity from atypical communication.

When to get more support

Treat **new or escalating distress** as a medical red flag until pain, infection, constipation, dental problems and sensory/environmental causes are excluded. Screen for **autistic burnout** under chronic load. Involve **specialist autism/learning-disability teams** and speech-and-language (AAC). Document an individualised **sensory, communication and reasonable-adjustments profile** and embed it in the care plan.

Gender note

High-support women are especially under-identified and under-served; their distress is even more often misread. Apply the same rigour to excluding pain and overload regardless of presentation or communication method.

About this guide

AI-assisted, derived from this project's research drafts — the **Monotropism** brief and the **Lifespan Practitioner & Health-Care guide** (Parts 3.5 & 4) plus the **Family guide** (Part 5, care homes & hospitals). Uses identity-first language. **Informational — not medical advice or a diagnostic tool.** Fit it to the individual.

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020); Kelly Mahler (2024, interoception/pain); AASPIRE AHAT; Autism Society (2025, meltdowns/shutdowns); Leicestershire Partnership NHS Trust (Autism Space). Full citations are in the source drafts.